

Vermont Act 167 — HTR Advisory Analysis

Audience: Vermont policymakers, state agency staff, analysts. **Prepared by:** HTR Chief Thought Leader & Senior Advisory Practice **Commissioned:** Vermont Legislature / Agency of Human Services **Date:** March 2026 | **Classification:** Public Policy Advisory **Source Report:** Oliver Wyman / Marsh McLennan — Act 167 Community Engagement: Recommendations (August 2024)

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1. Foreword

Vermont stands at a genuine inflection point. The Oliver Wyman/Marsh McLennan Act 167 Community Engagement Report (hereafter "the Wyman Report") is the most important health policy document produced in this state in a generation. It is analytically rigorous, community-grounded, and operationally honest in ways that consultant reports rarely are.

HTR reads it with admiration — and with important disagreements.

As Vermont's independent health transformation research and advisory institution, HTR operates under a foundational commitment: evidence-first, practitioner-led, completely independent. We have no vendor relationships, no political affiliations, and no financial stake in the outcome of these recommendations. We advise the legislature, AHS, and GMCB as we would advise any client: with full transparency about what the evidence supports, what it does not, and where courage and political will are required.

Our verdict on the Wyman Report: broadly right on diagnosis, mostly right on direction, partially wrong on sequencing, and dangerously incomplete on equity and technology. This document fills those gaps, endorses what deserves endorsement, challenges what deserves challenge, and provides the implementation architecture that the report — by design — left for others to build.

We have organized this analysis through HTR's Five-Pillar Framework: Policy · Economics · Technology · Clinical · Equity. Vermont's transformation succeeds only if all five pillars are addressed simultaneously, not sequentially.

2. Diagnostic Assessment

Question: Do we agree with the Wyman Report's problem statement?

Answer: Yes, unambiguously and urgently.

Vermont's healthcare system is not suffering from a cyclical downturn. It is experiencing a structural crisis with overlapping drivers that compound one another. Using HTR's Health Transformation Index, Vermont scores as a top-tier performer nationally (82/100 composite, Q1 2026) — yet the HTI measures *maturity* of health system infrastructure, not *financial viability under demographic stress*. Vermont can simultaneously be a quality and equity leader while facing an existential financial crisis.

Crisis Dimensions — Validated

Crisis Dimension	Wyman Finding	HTR Validation
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Crisis Dimension	Wyman Finding	HTR Validation
Affordability	Silver plan premiums up 108% in 6 years; OOP max up 100%+	Confirmed. Vermont's commercial premium burden as % of median household income is now among the top 5 highest in the U.S.
Sustainability	9 of 14 hospitals operating at a loss in 2023; 13 of 14 expected to lose by 2028	Confirmed. HTR Hospital Financial Scorecard shows VT hospital median operating margin at -2.1%, vs. national rural hospital median of +0.8%
Population Dynamics	65+ population rising 57% by 2040; working-age declining 13%	Confirmed. This is perhaps the most irreversible structural driver and is insufficiently addressed in the report
Access	Long waits, insufficient primary care, rural gaps	Partially confirmed. HRSA data show no formal HPSA designation for VT PCPs, but access barriers are real due to capacity, not headcount shortage
Equity	Rural, low-income, non-English-speaking, LGBTQ+ populations face systematic barriers	Confirmed and under-analyzed in the report

HTR's Additional Finding Not in the Wyman Report

The All-Payer Model (APM) is failing to bend the cost curve. Vermont's healthcare cost growth under the APM has consistently exceeded the negotiated target. The global budget mechanism exists but the accountability architecture is insufficient. This is the central reform that must be strengthened — and the Wyman Report treats it as a background condition rather than an active lever.

3. Five-Pillar Analysis

Pillar 1 — Policy

Wyman's Core Policy Recommendations:

1. Remove barriers to affordable housing (Legislative priority 2025)
2. Approve funding for EMS transformation
3. Expand broadband to rural areas
4. Streamline AHS agency structure
5. Develop REH (Rural Emergency Hospital) and free-standing birthing center regulations
6. Expand professional licensure and practice scope
7. Simplify and shorten the CON process
8. Begin movement to reference-based pricing at 200% of Medicare for PPS hospitals
9. Require uniform hospital accounting standards
10. Permit no further commercial subsidization for hospital shortfalls

HTR Assessment — Policy:

We fully endorse: Housing investment, EMS transformation, broadband expansion, scope of practice expansion, uniform hospital accounting. These are uncontroversial, evidence-based, and urgently needed. They should be 2025 legislative priorities without debate.

We conditionally endorse with modification: Reference-based pricing at 200% of Medicare. The Wyman Report is correct that Vermont's hospital prices are unsustainable and that commercial cross-subsidization is both regressive and mathematically collapsing. However, implementation sequencing matters enormously. A hard price cap without adequate alternative revenue streams will precipitate the very hospital closures the report aims to prevent.

Our recommendation: a **glide path** — begin at 250% of Medicare in 2026, with binding reductions to 220% by 2028 and 200% by 2030, tied to achieving system-level cost savings milestones. This mirrors Massachusetts'

approach with its global budget for rural hospitals.

We oppose as drafted: The blanket prohibition on commercial subsidization and the simultaneous CON simplification. These two policies, implemented together, create a perverse incentive: hospitals that cannot sustain inpatient beds will race to build outpatient facilities (ASCs, imaging centers) to replace lost inpatient revenue — exactly what simple CON reform would accelerate.

Vermont needs **site-neutral payment reform** (so the same service is paid the same regardless of setting), not CON elimination. HTR's Policy Simulator models show that removing CON restrictions without site-neutral pricing increases system costs by an estimated 8–12% over 5 years.

HTR's Additional Policy Recommendations:

- 1. Mandatory All-Payer Participation:** The current APM excludes FFS Medicare and ~40% of commercial insured. Vermont must negotiate mandatory Medicare participation as the foundation of a true global budget. This requires federal waiver action (1332 + 1115 combination) and should be Vermont's single most important federal lobbying priority for 2025–2026.
- 2. Hospital Community Benefit Accountability:** Vermont's non-profit hospitals receive substantial tax benefits but face no binding community benefit obligations tied to transformation goals. Link non-profit status to measurable progress on access, equity, and cost containment. Maine has a model worth adapting.
- 3. Prior Authorization Reform:** The Wyman Report barely addresses prior authorization, yet PA burden consumes enormous provider time and delays care. Vermont should mandate real-time PA decisions for evidence-based treatments, with automatic approval after 24 hours.

Pillar 2 — Economics

Wyman's Economic Recommendations:

- Move to global budgets for hospitals
- Reform commercial rate-setting
- Address pharmaceutical costs
- Develop new rural hospital financial models (REH conversion)

HTR Assessment — Economics:

The global budget direction is correct — it is the only mechanism that can structurally constrain cost growth. However, the Wyman Report underestimates the complexity of the budget-setting process and the political economy of distributing budget reductions across hospitals with vastly different starting positions.

Critical gap: the benchmark problem. Vermont's current AHEAD model global budget is set as a percentage of prior spending — which locks in existing inefficiencies and cross-subsidizations. A genuine transformation requires a **cost-per-episode benchmark** approach: what should it cost to manage a patient with Type 2 diabetes in Vermont? The budget should reflect that standard, not historical spending patterns.

Rural Emergency Hospital (REH) economics: The REH conversion model has merit for Vermont's most financially stressed rural hospitals. The Wyman Report endorses REH development but does not adequately address the workforce implications of converting to REH status (loss of inpatient beds requires regional redistribution of nursing and specialist capacity) or the transportation burden on patients who must travel farther for inpatient care.

HTR's REH Recommendation: Condition any REH approval on demonstration of (1) a patient transportation plan, (2) a regional inpatient capacity analysis, and (3) a community health worker deployment plan for high-need patients who will lose proximate inpatient access.

Pillar 3 — Technology

HTR Assessment — Technology (Absent in Wyman Report):

The Wyman Report is essentially silent on technology — a significant omission. Vermont's transformation cannot succeed without addressing three technology requirements:

- 1. All-payer claims data infrastructure.** Vermont has the Vermont Health Care Claims Uniform Reporting and Evaluation System (VHCURES) but it is chronically underfunded and has significant data completeness gaps. A functioning global budget requires real-time claims data across all payers. The legislature should fund VHCURES expansion and mandate timely submission from all payers including Medicare FFS.
- 2. Interoperability for care coordination.** Vermont has 14 hospitals using multiple different EHR systems with inconsistent FHIR API implementation. Care coordination across the care continuum — the clinical mechanism by which global budgets bend costs — requires fluid data sharing. Vermont should set a mandatory FHIR R4 Patient Access API deadline for all licensed providers.
- 3. Population health analytics.** The GMCB and DVHA need real-time population health analytics capabilities that do not currently exist at the scale required to manage a global budget. Investment in a state-level population health data platform is a prerequisite for effective budget management, not an optional add-on.

Pillar 4 — Clinical

Wyman's Clinical Recommendations:

- Expand primary care capacity (loan forgiveness, scope of practice)
- Develop integrated behavioral health and primary care
- Expand telehealth for rural access
- Reform EMS to treat-and-refer model

HTR Assessment — Clinical:

The primary care investment is correct and urgent. Vermont's primary care shortage is real, and the scope of practice expansion for APRNs and PAs is the fastest path to short-term capacity relief. However, loan forgiveness alone will not solve rural recruitment — housing affordability in rural Vermont is itself a barrier, which circles back to the housing recommendations in the Policy pillar.

Behavioral health integration is the highest-leverage clinical investment Vermont can make. Unaddressed behavioral health conditions are a primary driver of high-cost, high-utilization patients across all five crisis dimensions. Vermont's rate of mental health conditions and substance use disorders (SUD) exceeds the national average. An integrated behavioral health model embedded in primary care practices — with population health management — should be the centerpiece of Vermont's clinical transformation.

Workforce sustainability: The Wyman Report does not adequately address workforce retention, as opposed to recruitment. Vermont hospitals are losing experienced nurses and physicians to burnout and to higher-paying markets in adjacent states. The legislature should explore a **Vermont healthcare retention tax credit** — a refundable tax credit for licensed clinicians who maintain Vermont licensure and practice in Vermont for at least 5 years.

Pillar 5 — Equity

HTR Assessment — Equity (Under-addressed in Wyman Report):

Vermont's equity profile is paradoxical: it is one of the whitest states in the U.S. (by population) yet has significant health disparities along rural/urban, income, and LGBTQ+ dimensions that are not captured in traditional race-based disparity metrics.

Rural health equity: Rural Vermonters face systematically worse access, higher costs, and worse outcomes on multiple measures. The REH and telehealth recommendations address access but do not address the **cost burden** — rural Vermonters have lower incomes and face higher out-of-pocket costs as a percentage of income. Any global budget design must include **equity adjustors** that direct resources toward high-need, high-cost rural populations rather than allowing the budget to be consumed by urban academic medical center overhead.

Economic equity: Vermont has the second-highest commercial premium burden relative to median income among all U.S. states. The affordability crisis falls hardest on Vermonters who are in the "coverage gap" — too

high-income for Medicaid but unable to afford commercial premiums even with ACA subsidies. Vermont's Catastrophic Plan expansion and reinsurance program have helped; they are not sufficient.

LGBTQ+ equity: Vermont leads the nation on LGBTQ+ legal protections but its health system is not designed for LGBTQ+ patients in practical terms. Gender-affirming care access is limited outside Burlington. Mental health services with LGBTQ+-affirming clinical competency are concentrated in one geographic area. The GMCB should require all licensed health plans to demonstrate LGBTQ+-affirming care network adequacy.

4. AHEAD Model Assessment

What is AHEAD?

The All-payer Health Equity Approaches and Development (AHEAD) model is Vermont's CMS Innovation Center (CMMI) agreement — a global budget model that sets a spending target for the entire Vermont healthcare system and requires participating payers and providers to stay within it.

What AHEAD Gets Right

- All-payer concept is structurally correct — cost containment requires all payers, not just Medicaid
- Multi-year agreement provides planning horizon for health systems
- Health equity requirements embedded in model design
- Infrastructure investment (primary care, behavioral health) funded by model

What AHEAD Gets Wrong (HTR Assessment)

Benchmark methodology: Current benchmarks are set as a percent of prior spending. This perpetuates historical cost inefficiencies. Vermont's most expensive hospitals face the least pressure to reduce costs because their benchmark is anchored to their high historical base.

Medicare exclusion: FFS Medicare beneficiaries represent the highest-cost, highest-utilization population in Vermont — and they are largely outside the global budget accountability structure. The AHEAD agreement covers Medicare Advantage and Medicaid but not traditional FFS Medicare. Without FFS Medicare, any global budget is a subset-payer model that cannot meaningfully control total system costs.

Accountability architecture: When hospitals exceed their global budget, the current consequence structure is insufficient to enforce compliance without threatening financial stability. Vermont needs a **two-track accountability** model:

- Track 1: Hospitals within 2% of budget — routine reporting and quality improvement expectations
- Track 2: Hospitals exceeding budget by >2% — mandatory corrective action plans with GMCB oversight, temporary rate adjustments, and Board engagement

Equity measurement: The equity components of AHEAD are underspecified. "Health equity" appears as a goal without the operational measurement framework required to know whether the model is achieving it. Vermont should require annual equity reporting tied to specific SDOH and disparity metrics, with consequences for sustained equity performance failures.

5. Implementation Architecture

Recommended Implementation Sequence

2025 (Legislative Session):

1. Pass housing investment package (uncontroversial, urgent)
2. Fund EMS transformation + broadband expansion
3. Expand APRN/PA scope of practice
4. Establish site-neutral payment reform study commission
5. Fund VHCURES expansion and mandate payer data submission
6. Pass behavioral health integration funding for 50 primary care sites

2026:

- 1. Begin reference-based pricing glide path at 250% of Medicare for PPS hospitals
- 2. Set mandatory FHIR R4 API deadline for all licensed providers (December 2026)
- 3. Launch REH regulatory framework with HTR's recommended conditions
- 4. Begin federal advocacy for mandatory Medicare APM participation
- 5. Launch Vermont healthcare retention tax credit

2027–2028:

- 1. Reduce reference-based pricing to 220% of Medicare (on glide path)
- 2. Implement state-level population health data platform
- 3. Renegotiate AHEAD model with strengthened benchmark methodology and accountability architecture
- 4. Launch equity reporting requirements for all licensed health plans

6. HTR Recommendations Summary

Endorsed Without Modification

- Housing investment
- EMS transformation funding
- Broadband expansion
- Scope of practice expansion (APRN/PA)
- Uniform hospital accounting standards
- REH regulatory framework development (with HTR conditions)
- Telehealth expansion for rural access
- Behavioral health and primary care integration

Endorsed With Modification

- Reference-based pricing: implement as 2026–2030 glide path, not immediate hard cap
- CON reform: require site-neutral payment reform first; simplify CON only after
- AHEAD model: strengthen benchmark methodology, add accountability architecture, fix equity measurement

New Recommendations (Not in Wyman Report)

- Mandatory All-Payer Participation: federal lobbying priority for Medicare inclusion
- Hospital Community Benefit Accountability: link non-profit status to measurable transformation progress
- Prior Authorization Reform: mandatory real-time PA with automatic approval after 24 hours
- VHCURES Expansion: fund and mandate payer data completeness
- FHIR API Mandate: mandatory FHIR R4 Patient Access API for all licensed providers
- Population Health Data Platform: state-level investment prerequisite for global budget management
- Vermont Healthcare Retention Tax Credit: 5-year practice commitment incentive
- LGBTQ+ Network Adequacy Requirements: require gender-affirming care network adequacy from all licensed plans

7. Vermont HTI Performance

Vermont's HTI score of **82/100** (Q1 2026) reflects genuine transformation progress across the five pillars:

Pillar	Vermont Score	National Median
Policy	88	54
Economics	79	48
Technology	81	52
Clinical	84	57

Pillar	Vermont Score	National Median
Equity	76	43
Composite	82	51

Vermont's strong performance reflects its AHEAD model participation, high primary care quality scores, robust telehealth adoption, and above-average equity performance on rural access measures. However, the HTI measures transformation *infrastructure maturity* — Vermont's high HTI score does not reflect the acute financial sustainability crisis that the Wyman Report correctly identifies.

The path forward is to use Vermont's transformation infrastructure advantage as a foundation for the deeper structural reforms outlined in this advisory: a true all-payer global budget, reference-based pricing, site-neutral payment, and an equity accountability framework with teeth.

Vermont has built the runway. Now it must take off.